

إسم المريض رباعياً : امل عبدالحكيم محمود مسلم
المحافظة : القاهرة الحي / المدينة : المعادي
رقم البطاقة : 26606161400401 رقم التليفون : 01064590558
العمر(تاريخ الميلاد) : 16-06-1966 المهنة : ربه منزل
العنوان : ش حسني
تاريخ تسجيل الإستمارة : 07-10-2026

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص النهائي : ورم خبيث بالمستقيم
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6184] قسم العلاج : الاورام
التعليق على التشخيص : -----

الاسم: د/محمد كيلاي الوظيفة: _____ التوقيع: _____
 الاسم: د/ندي حسن الوظيفة: _____ التوقيع: _____
 الاسم: د/نرمين رمضان الوظيفة: _____ التوقيع: _____

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفعة
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة معاينة المريض بالمستشفى قبل الصرف.

----- /d ----- /d ----- /d.l ----- /d.l

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 جهة العلاج :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

Name: أمل عبدالحكيم محمود مسلم
National ID: 26606161400401



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :

PM 01:17 05-07-2026

تاريخ وساعة الدخول :

---:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

A patient of colorectal cancer for Supportive and palliative Treatment

الوقت :

---:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاثمين

جمهورية مصر العربية
وزارة التعليم العالي والبحث العلمي
مجلس أمناء الجامعات

أمناء
عبدالحكيم محمود مسلم حسن

٩٦ ش ١٠
المعالي - القاهرة

٢ ٦٦ ٠ ٦ ١ ٦ ١ ٤ ٠ ٠ ٤ ٠ ١

LW1263672



Name: امل عبدالحكيم محمود مسلم		
MR. : 12939		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
نتشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 60-year-old individual with a rectosigmoid adenocarcinoma, located 10 cm from the anal verge, histologically graded as moderately differentiated, grade 2. The patient received three cycles of XELOX, and concurrent chemoradiotherapy was completed on 30 May 2026. A follow-up PET scan demonstrated mild metabolic regression of the rectosigmoid mass, but identified a new hepatic focal lesion measuring 1.6 cm with SUVmax 7.8, as well as progression of perirectal and presacral metastases. Molecular testing revealed KRAS and BRAF mutations and microsatellite instability (MSI) as discussed in the multidisciplinary tumor board. The patient has completed 3 cycles of XELOX. The patient was planned for cycle 2 of FOLFIRI; however, the complete blood count was pending, with an absolute neutrophil count of $1.34 \times 10^9/L$, prompting a one-week postponement of therapy while awaiting KRAS results. The patient is currently at cycle 2 of FOLFIRI. The total lymphocyte count was $2.7 \times 10^9/L$ and neutrophil count $0.8 \times 10^9/L$; therapy was postponed for one week, and consideration was given to administering 10 % dose reduction and prophylactic Neupogen.		
تحريراً في ,		
	الوقت :	التاريخ : 20-09-2026
حنان عزت شفيق ناودروس		الطبيب المعالج :
يعتمد ,		

GUSTAVE ROUSSY INTERNATIONAL REGISTRE		Date Of Submission:		Date Of MDT: 08/08/2024 10:15:01	
		<i>about 19</i>			
General GRI-E MDT form					
Patient Name:	ال دكتور محمد		Age:	60 Y	
Sex:	☐ Male ☐ Female		Comorbidity:	D hypertensive	
ID No.:	12919		Telephone:	01127551323	
Family History:	26505151400401		Marital Status:	☐ Married ☐ Divorced ☐ Single	
Mensesual Status:			Reason of MDT:		
			MDT name:		
MDT Decision					
Diagnosis:					
Notes:					
History:	60 YEARS OLD LADY rectosigmoid mass 10 cm from anal verge. HPE mod diff adenocarcinoma grade 2. received 3 cycles of XELOX the concurrent ch/rx ended 30/5/20. new pet scan showed mild metabolic regression of the rectosigmoid mass newly developed N/L 1. CH size MAX 7.8 and progression perirectal and presacral met progression for KRAS, braf, MSI mdt				
Investigations:					
Recommendations:	for KRAS, braf, MSI mdt for second line FOLFIRI avastin versus TKI according to kras				
MDT DECISION:	for KRAS, braf, MSI mdt for second line FOLFIRI avastin versus TKI according to kras				
MDT Decision					
Dr. Hanan Elzot					

Patient Name: **امل عبد الحكيم محمود**

Age/Sex: **60Ys/ F**

Request Date: **21-1-2026**

Printed IN: **25 -1 -2026**

Referred by Prof. Dr: **Amr Mostafa**

PATHOLOGY REPORT

Clinical history: colonic mass

Operation: Colonoscopy

Gross description: Specimen was received fixed in formalin revealed tiny tissue fragments each about 0.3x0.3 cm, totally submitted .

Microscopic description :

~~Section showed invasive epithelial glandular growth formed of irregular malignant haphazardly infiltrating glands with cribriform formation, lined with highly atypical cells with pleomorphic nuclei.~~

DIAGNOSIS: Colonic biopsy:

COLONIC MODERATELY DIFFERENTIATED ADENOCARCINOMA G II .

Signature:

Doaa A. El-Afy; MD

Colonoscopy Report

Name: أمل عبدالحكيم محمود
Date: Tuesday 20, January 2026

ID: 38694
Age: 60

أمل عبدالحكيم محمود
16795

Referring Physician:

Dear Prof.

Thank you for referring the

Indications

Changing bowel habits.

Medications

Propofol

Instrument

OLYMPUS CF 260 L

Findings:

BBPS (7/9)

Colonoscopy was done up to cecum with terminal ileum intubation revealed large circumferential mass , easily to bleed on touch , with ulcerations at about 10 cm from the anal verge and about 5cm in length causing narrowing of the lumen , the scope was passed with difficulty , multiple biopsies were taken for HPE.
With retroflexion of the scope at the anal canal revealed two cords of small sized internal haemorrhoids.

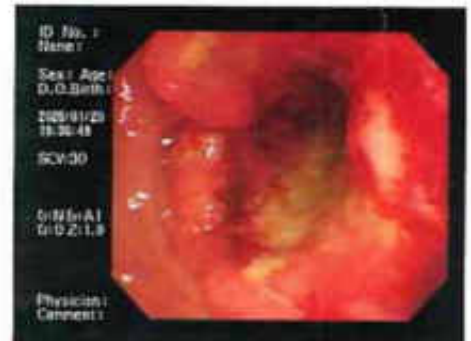
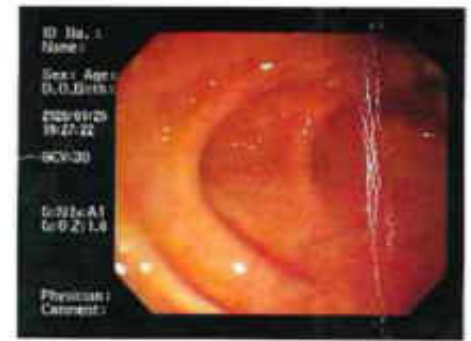
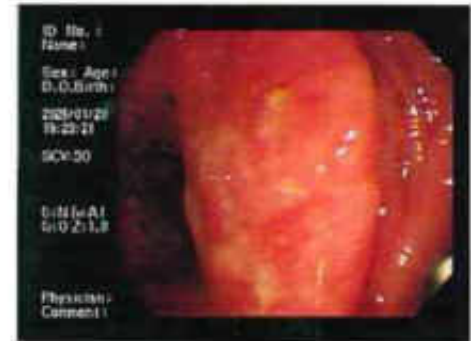
Conclusion

Rectosigmoid mass , most probably neoplastic for histopathological examination.
Internal haemorrhoids.

Recommendation

Check biopsies result ,
PET CT scan.

د. أمرو مصطفى
استشاري أمراض الجهاز الهضمي
والكبد والكلى والأمعاء

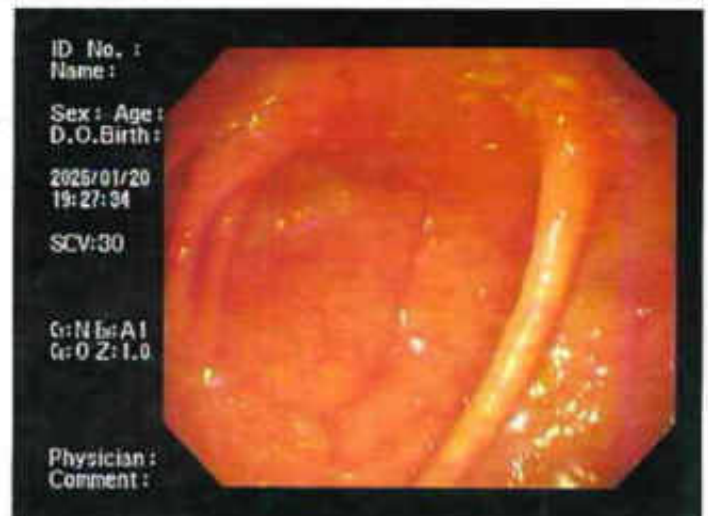
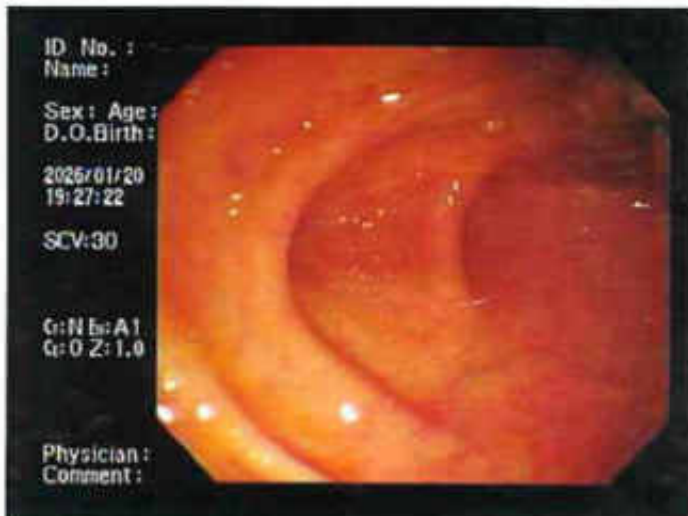
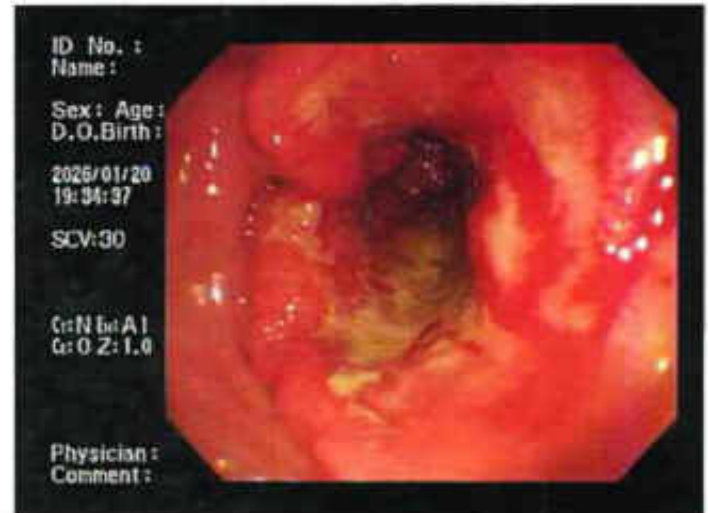
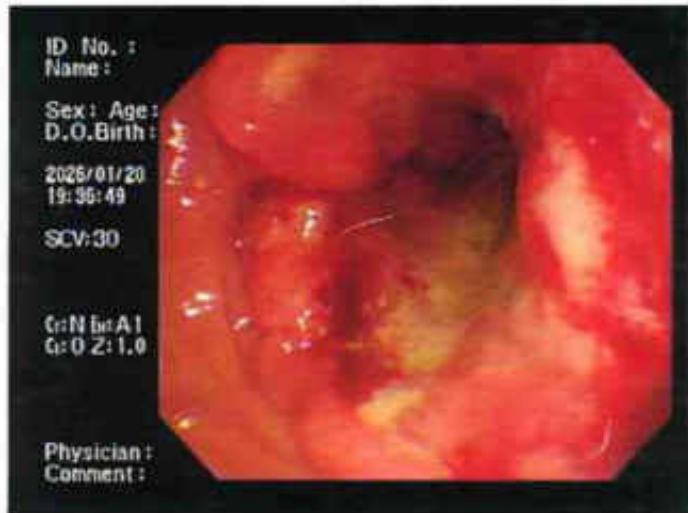
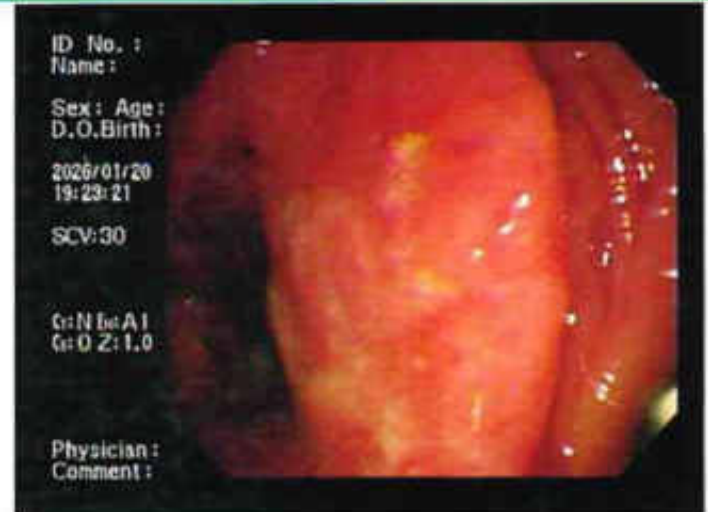
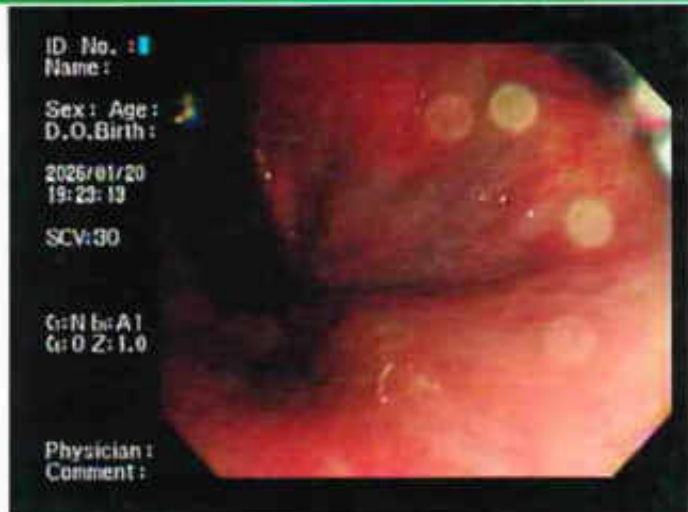


Signature Amr Moustafa

Colonoscopy Report

Name: أمل عبدالحكيم محمود
Date: Tuesday 20, January 2026

ID: 38694
Age: 60



شماره : 01152360225
شماره : 01211546666
شماره : 01022167784
شماره : 01099499592

الدقي: 107 شارع التحرير - الدقي - الجيزة
مصر الجديدة: 20 شارع بيل الوفاة - هليوبوليس - القاهرة
6 أكتوبر: 425 المحور المركزي - الحي الأول - المجاورة الثانية - 6 أكتوبر - الجيزة
حلوان: 46 شارع منصور تقاطع شارع شريف امام محطة مترو حلوان - حلوان - القاهرة

Visit Number:	61126517456	Client Name:	
Patient Name:	Amal Abdelhakeem Mahmoud	Referred By Prof:	
Age / Sex:	60 year / Female	Exam Date:	15 Jul 2026 00:00
Patient ID:	2140144159 Acc: 2044353	Printed:	7/16/2026 6:41:49 PM

PT

FOLLOW UP WHOLE BODY PET/CT EXAMINATION

HISTORY AND CLINICAL DATA:-

PATIENT DATA:-

- Female patient, 60 years old, Weight: 92 Kg, Blood Glucose level: 179 mg/dl.

RADIOPHARMACEUTICAL:-

- F-18-FDG 5.5 mCi injected I.V.

CLINICAL BACKGROUND:-

- Reported case of colo-rectal carcinoma, received 3 CTH sessions, last one was 3/2026. And received RTH, last session 1.5 month ago.

PURPOSE OF THE STUDY:-

- Assess response to therapy.

TECHNIQUE:-

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~2.0.
- Liver showed max. SUV~2.6.

REPORT:-

- ✓ Compared to the last study done on 18/4/2026.
- ✓ Review of the PET, CT and the fused images revealed:-

I) Loco-regional:

- Mild metabolic regression despite further activity extension of the previously seen FDG avid recto-sigmoidal segmental non-uniform neoplastic mural thickening, **extending now more towards the sigmoid colon** & reaches 1.8 cm in max. thickness; with max. SUV~11.1 compared to max. SUV~ 16.2 in last study.
- Still noted small mild FDG avid peri-colic fat stranding; with associated peri-rectal & pre-sacral lymph nodes; with SUVmax 4.8 compared to SUVmax 3.6 (mild metabolic progression) and measure up to 2 cm.

II) Liver:

Visit Number:	61126517456	Client Name:	
Patient Name:	Amal Abdelhakeem Mahmoud	Referred By Prof:	
Age / Sex:	60 year / Female	Exam Date:	15 Jul 2026 00:00
Patient ID:	2140144159 Acc: 2044353	Printed:	7/16/2026 6:41:49 PM

PT

- Newly developed FDG avid two hepatic focal lesions seen at the caudate lobe, with SUVmax 7.8 and measuring up to ~1.6 cm.

III) THE REST OF THE BODY:

- No other FDG avid lesions seen all over the rest of the surveyed body.
- No Ascites. No FDG avid peritoneal soft tissue lesions.
- Average sized spleen showing normal FDG uptake with no metabolically active FDG avid focal lesions.
- Normal bone marrow FDG uptake. No FDG avid lytic/sclerotic osseous lesions.
- Clear both lungs. No FDG avid pulmonary masses or nodules.
- No FDG avid adrenal or pancreatic masses.

SIDE FINDINGS:

- C4/C5 right facet arthropathy.
- Mild spondylosis.
- Elastofibroma dorsi.
- Aorto-iliac atheromatous mural calcifications.
- Large bilateral elasto-fibroma dorsi.
- Small hiatus hernia.

IV) The rest of PET / CT study appears normal.

- Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.

CONCLUSION:

- Follow up PET-CT study for a reported case of colo-rectal carcinoma showing:
 - ✓ Mild metabolic regression despite further activity extension regarding the FDG avid recto-sigmoid malignant mural thickening.
 - ✓ Mild metabolic progression of the FDG avid peri-rectal/pre-sacral metastatic LNs.
 - ✓ Newly developed FDG avid metastatic hepatic focal lesions.

Prof. Ahmed Wafaie MD, FRCR
Dr. Aya Mustafa Mussa, MD

Patient Name	امل عبدالحكم محمدر	Case No.	357712
Sex/Age	Female 60 Y	Patient No.	25820
Registered Date	12/8/2026 01:51:35 PM	Referred By Dr.	Out Patient
Validation Date	17/8/2026 12:14:34 PM	Branch	Gustave Roussy
Reporting Date	07/9/2026 12:40:15 PM	Pathology No.	SR-934/26

Pathology Report

Clinical Data: Colonic mass.

Nature of Specimen: Referred block (Colonoscopy biopsy).

Gross Examination:

One referred block labelled by patient Arabic name.

Microscopic Examination:

- Histopathological examination reveals fragments of colonic mucosa involved by invasive moderately differentiated adenocarcinoma formed of malignant glandular structures lined by highly pleomorphic malignant columnar cells with high N/C ratio and numerous mitotic figures. The tumor cells are surrounded by moderate number of TILs and stroma showing desmoplastic reaction.

Diagnosis:

Colonic mass, Referred block (Colonoscopic biopsy):

- Invasive moderately differentiated adenocarcinoma, Grade 2.

Signature

Dr. Walaa Ibrahim Chonaim, MD.



RAS-BRAF Molecular Report

Name: Amal Abdel Hakim Mahmoud

Age: 60 years.

Clinical data: Invasive moderately differentiated adenocarcinoma grade 2

Specimen type: Referred Block

Date (Received): 18 / 08 / 2026

Date (reported): 23 / 08 / 2026

Lab. Number: SR-934/26

Referred by:

Indication: Predictive test for response to therapy.

Gene	Result	Mutation Detected
KRAS	Mutant	Mutation detected in Exon 2 – G12D
NRAS	Not-Tested	
BRAF	Not-Tested	

Interpretation & Comment

Based on the current results This patient should not be considered for anti-EGFR monoclonal antibody targeted therapy because as per the NCCN guidelines Colorectal tumors with any known KRAS mutation (exon 2, 3, 4) or NRAS mutation (exon 2, 3, 4) are associated with lack of clinical response to anti-EGFR.

Test Background

Approximately 40% of patients with mCRC harbor an activating KRAS & NRAS mutations in 4% of the patients. The outcome of patients with RAS mutations is worse compared to RAS wild type (RAS wt) cases. However, KRAS & NRAS mutations also have distinct impacts on survival in mCRC; the prognosis of the aggressive NRAS mutant molecular subset is poor.

According to National Comprehensive Cancer Network guidelines (NCCN; Version 1.2020) for metastatic colorectal cancer (mCRC), expanded RAS panel for activating mutation of KRAS & NRAS is crucial at time of diagnosis to select the patients who are likely to benefit from anti-EGFR therapies.

Test Description

Tissue sections were reviewed by a pathologist and relevant tumor was selected for analysis.

- DNA was isolated and amplified by quantitative polymerase chain reaction (qPCR) using NRAS and KRAS CE-IVD kits according to manufacture protocol. results interpreted according to the manufacture user guide. Mutation nomenclature was performed considering the following reference sequences: NM_004985.4 (KRAS) and NM_002524.4 (NRAS gene).
- BRAF V600E mutation specific Competitive allele specific TaqMan assay along with its reference wildtype assay were performed for each sample to accurately interpret the results.



Disclaimer:

- This test reported herein was performed at an external reference lab (57357 laboratory), verified by Biotechnology Molecular Interpretation Clinico-pathologic Interpretation

Sarah Sobhy, M. Sc., MBA
Molecular lab. Section Head

Abdelrahman Mohamed
Molecular Pathology lab Specialist

Doaa Hassan, FEEB
Pathology Consultant

Marwa Magdy, FEEB
Pathology Consultant

Mohamed Tamer
Molecular pathology specialist

Mohamed Khalifa, M. Sc.
Molecular pathology lab. Technical advisor



Microsatellite Instability Testing Report

Name: Amal Abdel Hakim Mahmoud

Age: 60 years.

Clinical data: Invasive moderately differentiated adenocarcinoma grade 2

Specimen type: Referred Block

Date (Received): 18 / 08 / 2026

Date (reported): 25 / 08 / 2026

Lab. Number: SR-934/26

Referred by:

Indication:

Predictive test for response to therapy.

Result

Microsatellite Stable (MSS)

Interpretation & Comment

In the absence of microsatellite instability, are unlikely to obtain clinical benefit from Immunotherapy check points inhibitors regimens.

Test Background

MSI screening test looks for changes in the DNA sequence between normal tissue and tumor tissue and can identify whether or not there is high amount of instability, which is called MSI-High. Result could be MSI high, low or stable, MSI stable are likely to benefit from standard chemotherapeutic regimens. But MS-LOW or MS-HIGH cases are not expected to obtain clinical benefit from treatment with standard chemotherapeutic regimens. Many patients with MSI-H tumors have had a positive response to immunotherapy treatments (or immune-checkpoint therapies).

Test Description

Method: FFPE DNA Extraction: DNA was isolated from neoplastic tumor cells

- Microsatellite Analysis: A total of 5 polymorphic markers are used in the interpretation of three levels of MSI, stable (MSS), Low (MS-LOW) or High (MS-High). The markers used in this analysis included: BAT25, BAT26, NR21, NR24 and NR 27. Two control markers are also included for quality control purposes.

- Bioinformatics analysis: Resulted files were subjected to analysis pipeline for MSI stability detection using True mark MSI analysis software, ThermoFisher.

- Method Limitation: In the absence of the patient specific control sample (Normal Tissue or Blood). This method able to detect microsatellite instability only in sample at allele frequency not less than 20% by detecting the emergence of any novel allele with a different repeat length compared to the reference sequence.



Assem Awad

Molecular Pathology Specialist

Mohamed Khalifa, M. Sc.

Molecular pathology lab. Technical advisor



GUSTAVE / ROUSSY /

INTERNATIONAL EGYPT

Dr. P. 9-7

Date Of Submission:

Date Of MDT: 09/07/2026 15:35:18

General GRI-E MDT form

Patient Name :		Age :	
Sex : ☐ Male ☐ Female جنس ذكر / أنثى		Comorbidity :	
ID No. :	12939	Telephone :	01127353323
Family History :	26606161400401	Marital Status :	☐ Married ☐ Divorced ☐ Single
Menopausal Status :	☐	Reason of MDT :	☐
MDT name::		☐	
Medical Report			
Diagnosis :			
Notes :			
History:			
60 YEARS OLD LADY rectosigmoid mass 10 cm from anal verge. HPE mod diff adeno ca grade 2. received 3 cycles of XELOX the concurrent ct/r ended 30/5 /26. new pet scan showed mild metabolic regression of the rectosigmoid mass newly developed HFL 1.6 CM SUV MAX 7.8 and progression perirectal and presacral met progression for KRAS , braf, MSI mdT			
Investigations:			
Recommendations:			
for KRAS , braf, MSI mdT for second line FOLFIRI avastin versus TKI according to kras RAS MUTANT for KRAS MUTANT for second line FOLFIRI / avastin			
MDT DECISION:			
Signature	Hematology	Pediatric	Bone marrow (transplant)
Dr Hanan Ezzat	☐	☐	☐

GUSTAVE ROUSSY INTERNATIONAL EGYPT		Date Of Submission:		Date Of MDT:	
				08/09/2026 10:15:01	
General GRI-E MDT form					
Patient Name :	أمل عبدالحكيم محمود مسلم		Age :	60 Y	
Sex :	☐ Male ☒ Female		Comorbidity :	D hypertensive	
ID No. :	12939	Telephone :	01127353323		
Family History :	26606161400401	Marital Status :	☐ Married ☐ Divorced ☐ Single		
Menopausal Status :		Reason of MDT :			
		MDT name::			
Medical Report					
Diagnosis :					
Notes :					
History:	60 YEARS OLD LADY rectosigmoid mass 10 cm from anal verge HPE mod diff adeno ca grade 2 received 3 cycles of XELOX the concurrent ch/r/t ended 30/5 /26 new pet scan showed mild metabolic regression of the rectosigmoid mass newly developed HFL 1.5 CM sup MAX 7.8 and progression perirectal and presacral met progression for KRAS , braf, MSI mdt				
Investigations:					
Recommendations:	for KRAS , braf, MSI mdt for second line FOLFOX avastin versus TKI according to kras				
MDT DECISION:	for KRAS , braf, MSI mdt for second line FOLFOX avastin versus TKI according to kras				
Signature	Hematology	Pediatric	Bone marrow transplant		
Dr Hanan Ezzat					

Colonoscopy Report

Name: امل عبدالحكم محمود

ID: 38694

Date: Tuesday 20, January 2026

Age: 60

Referring Physician:

Dear Prof.

Thank you for referring the

Indications

Changing bowel habits.

Medications

Propofol

Instrument

OLYMPUS CF 260 L

Findings:

BBPS (7/9)

Colonoscopy was done up to cecum with terminal ileum intubation revealed large circumferential mass , easily to bleed on touch , with ulcerations at about 10 cm from the anal verge and about 5cm in length causing narrowing of the lumen , the scope was passed with difficulty , multiple biopsies were taken for HPE. With retroflexion of the scope at the anal canal revealed two cords of small sized internal haemorrhoids.

Conclusion

Rectosigmoid mass , most probably neoplastic for histopathological examination.

Internal haemorrhoids.

Recommendation

Check biopsies result.

PET CT scan.

د. أمينة محمد مصطفى
رئيسة قسم المناظير
مصر سكوب

امل عبدالحكم محمود
16795

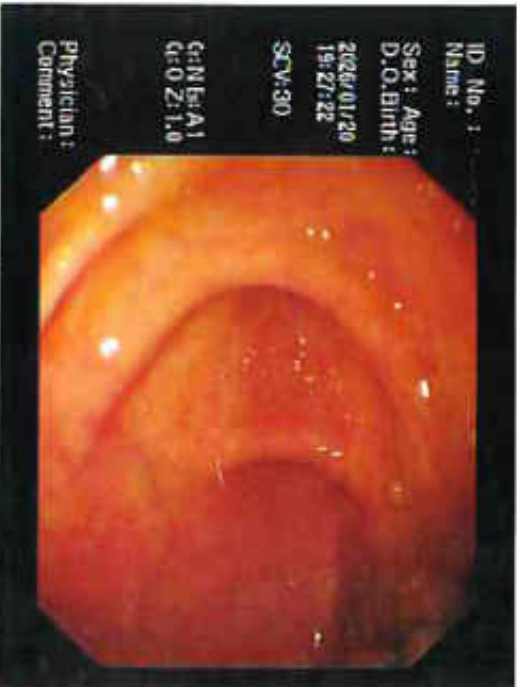
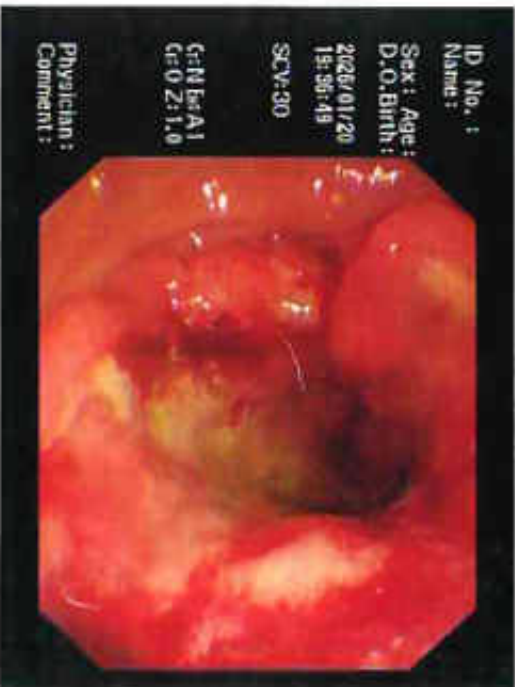
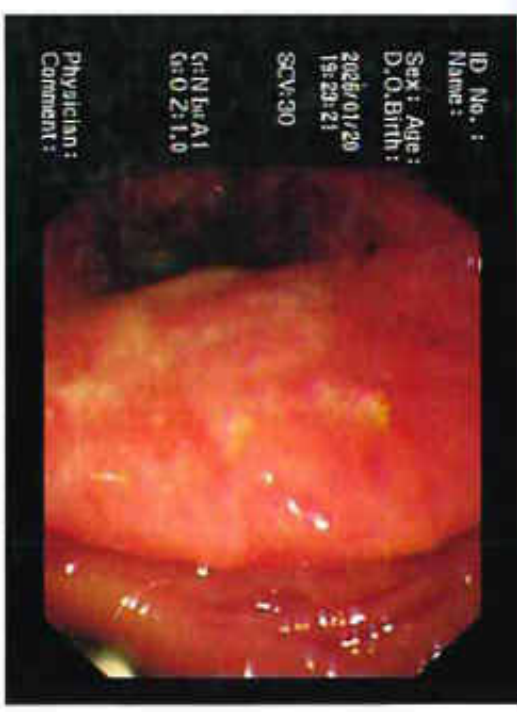


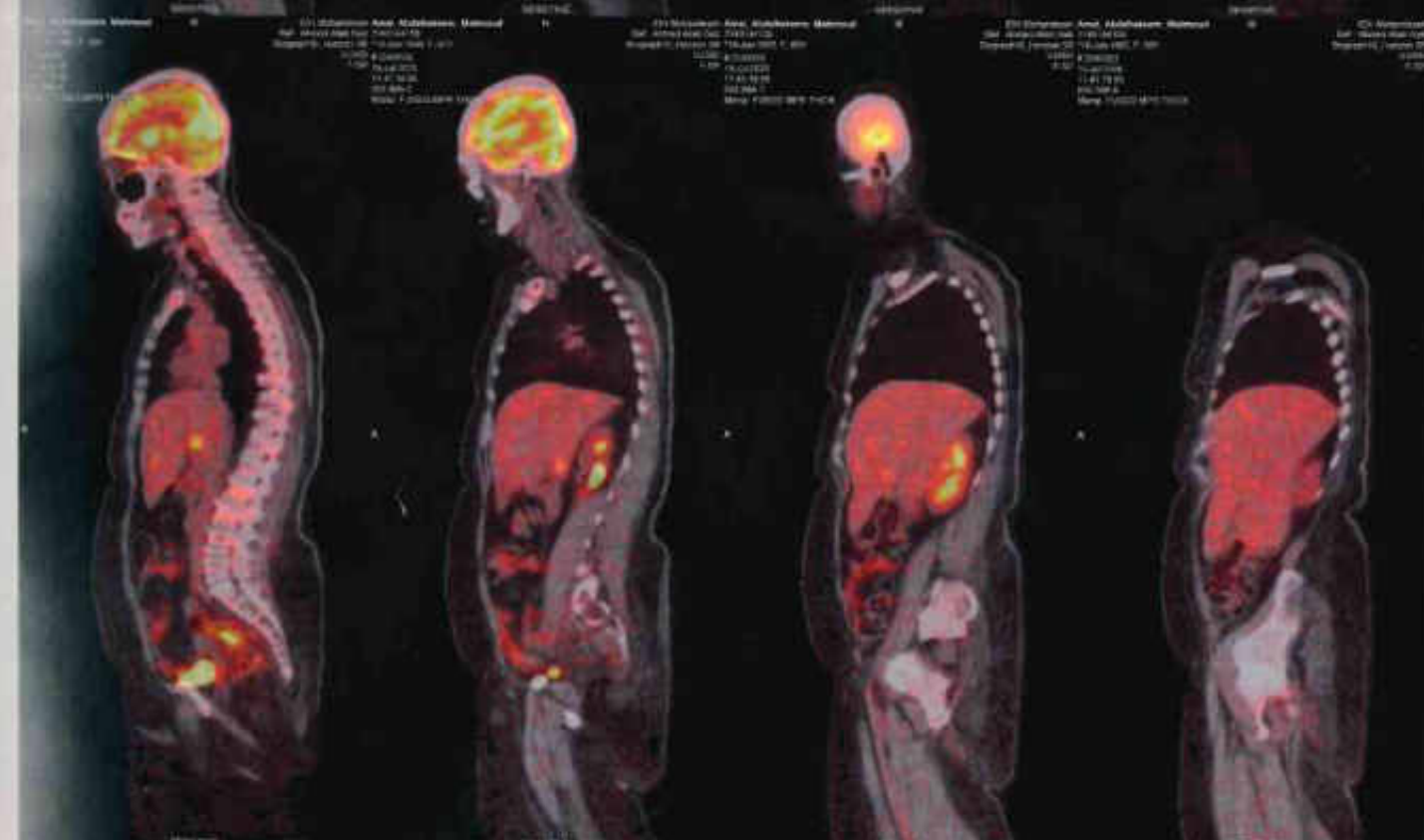
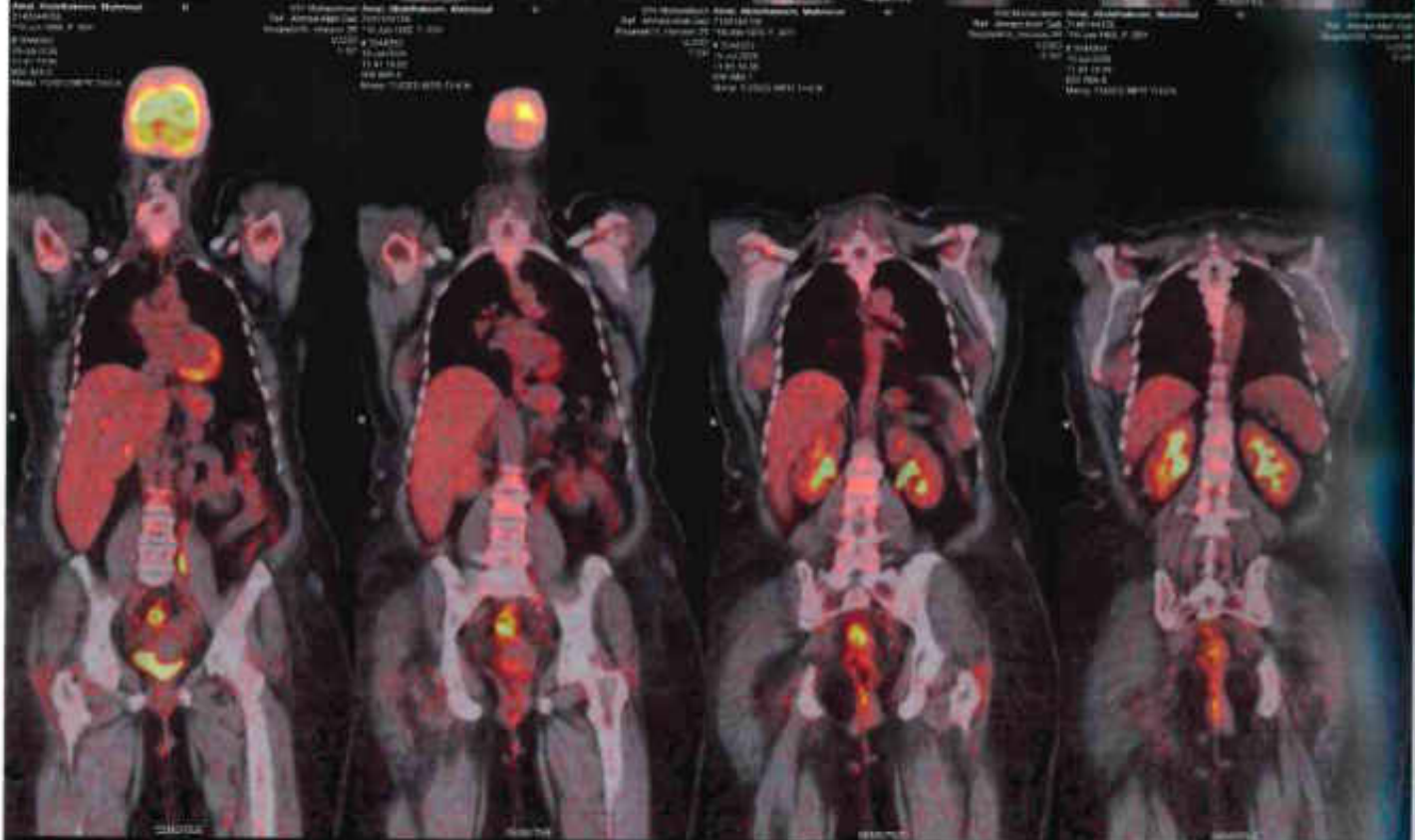
Signature Amr Moustafa

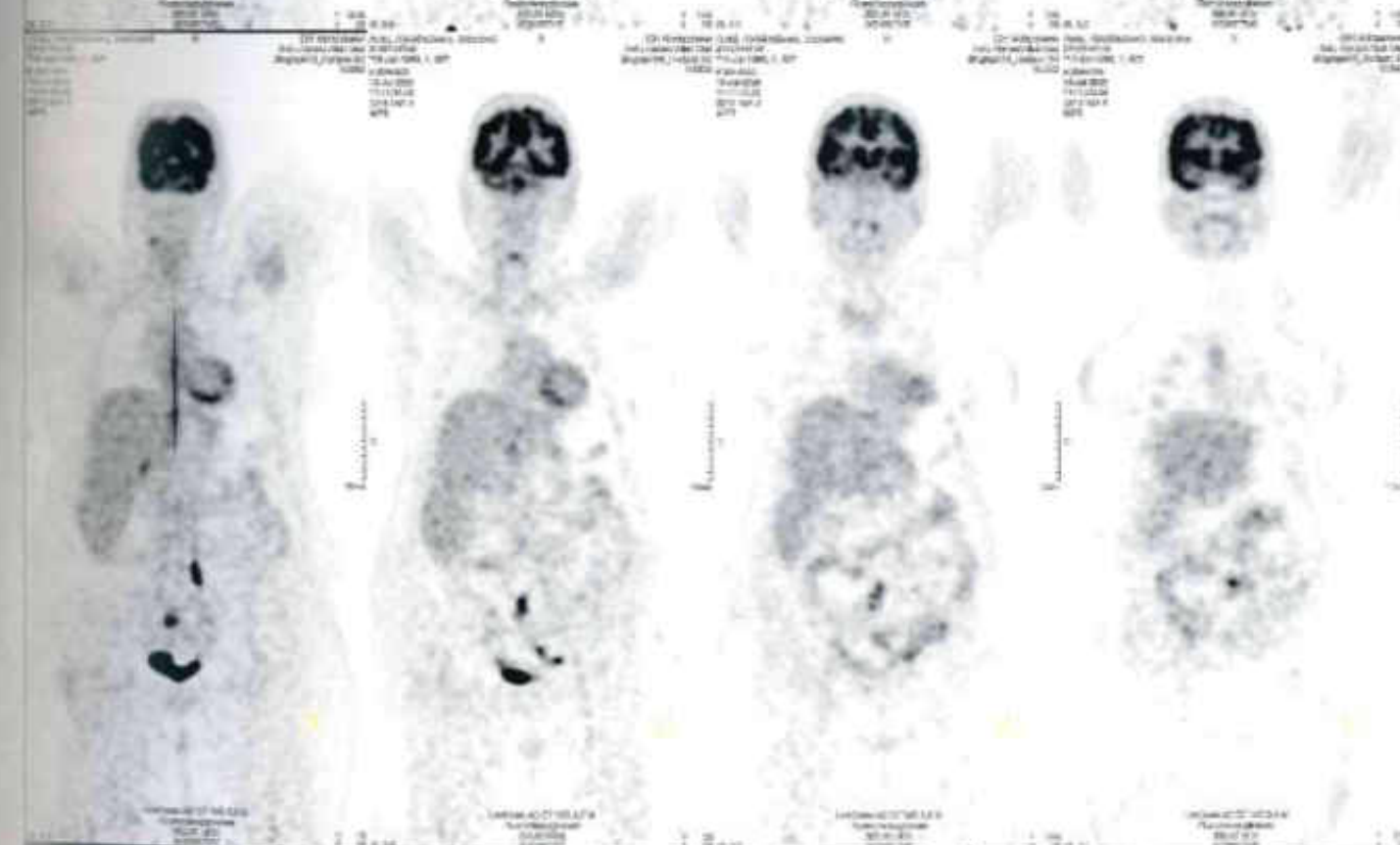
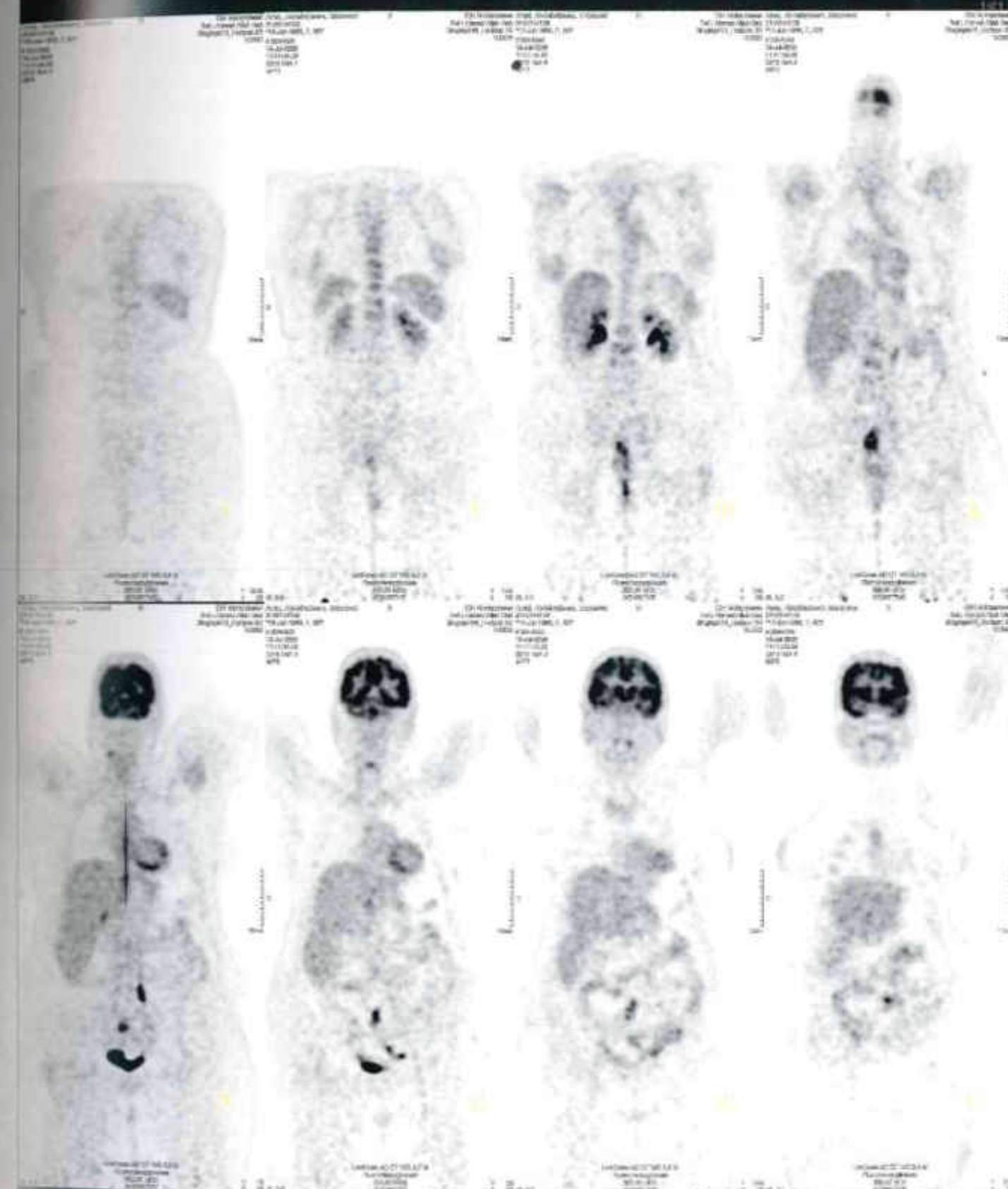
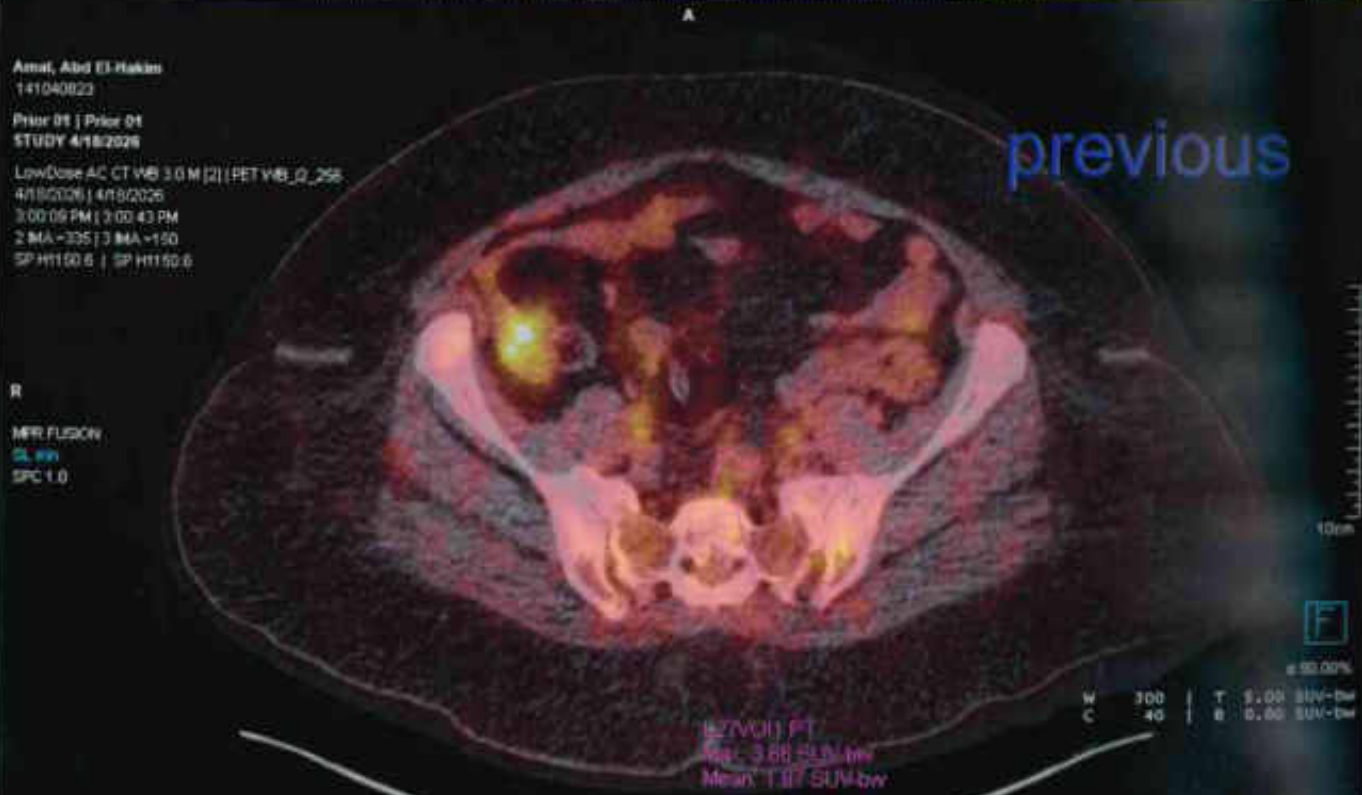
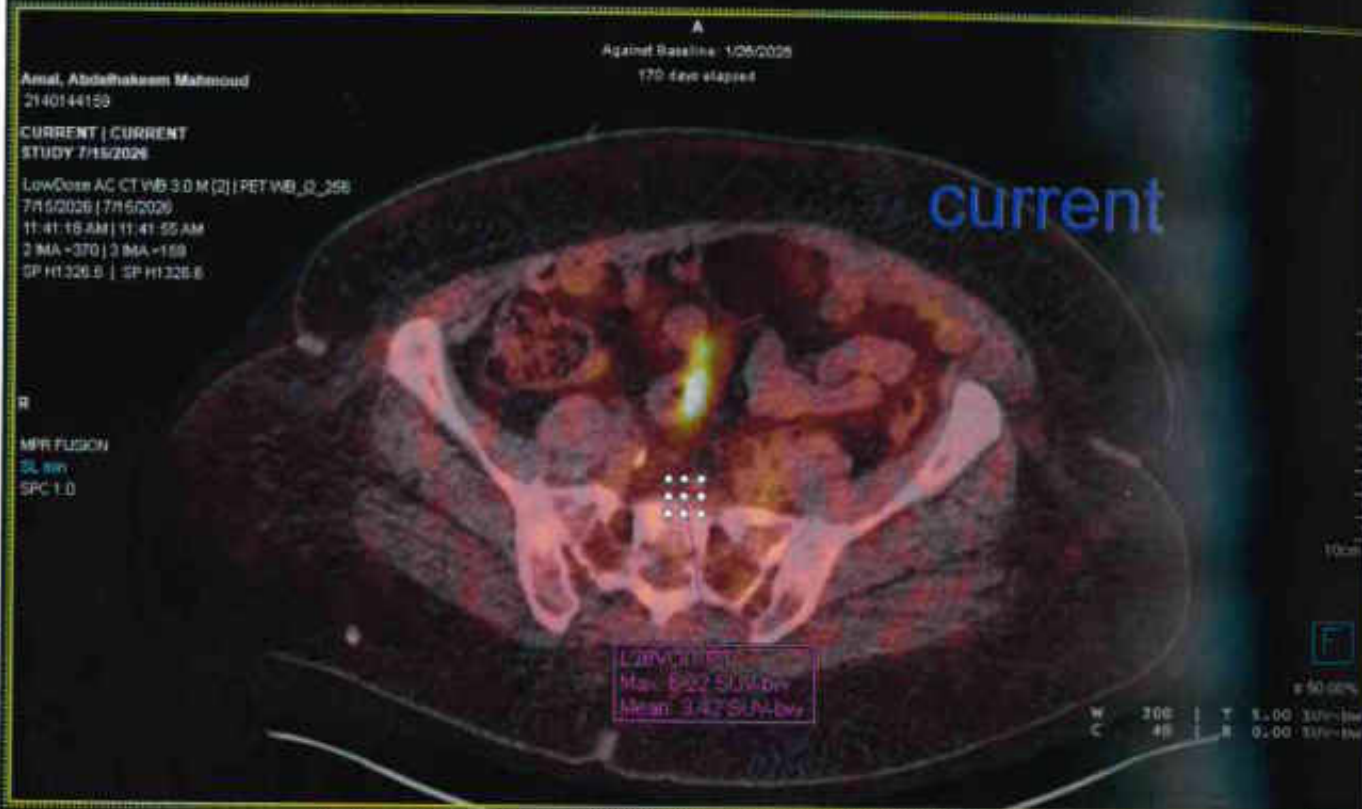
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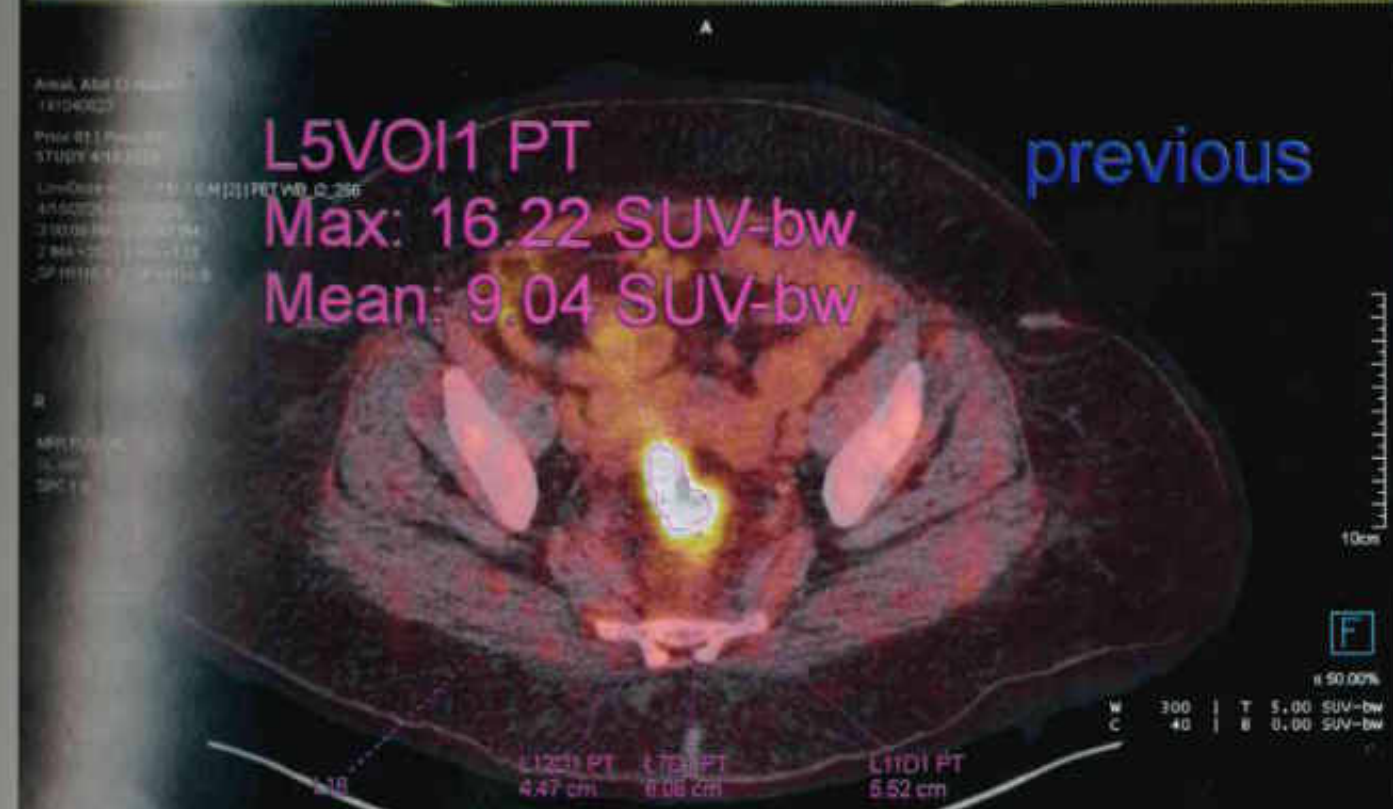
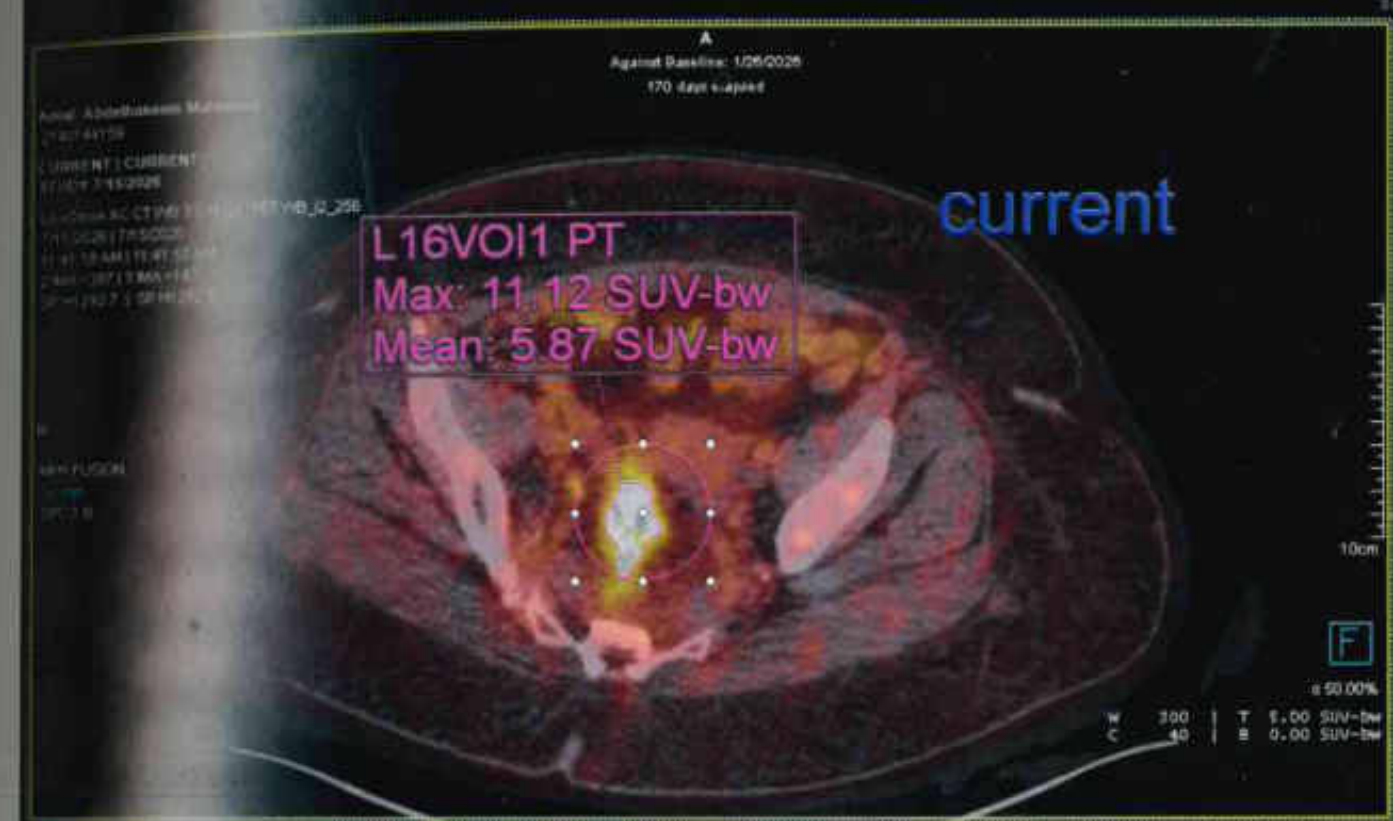
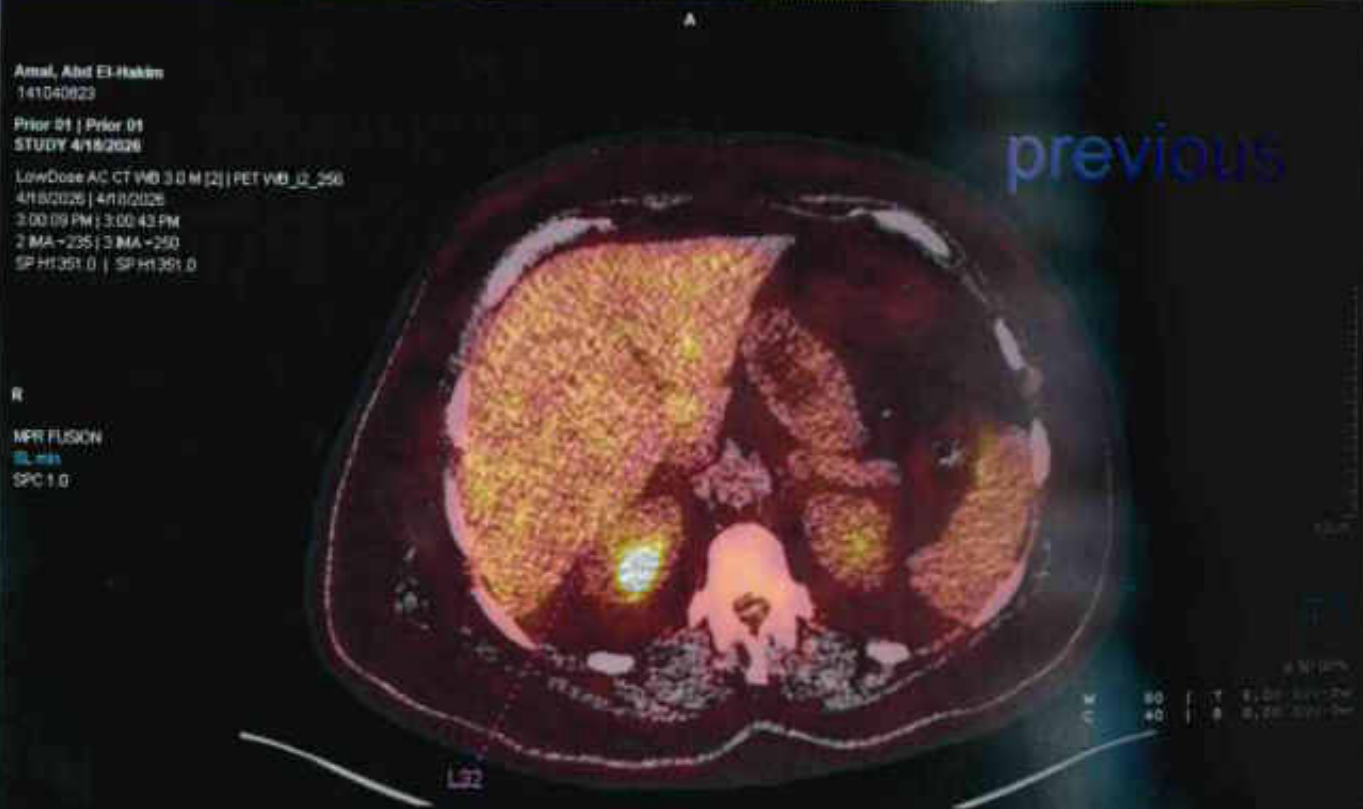
Name: **أحمد عبد الحليم محمد**
Date: Tuesday 20, January 2026

ID: 38694
Age: 60







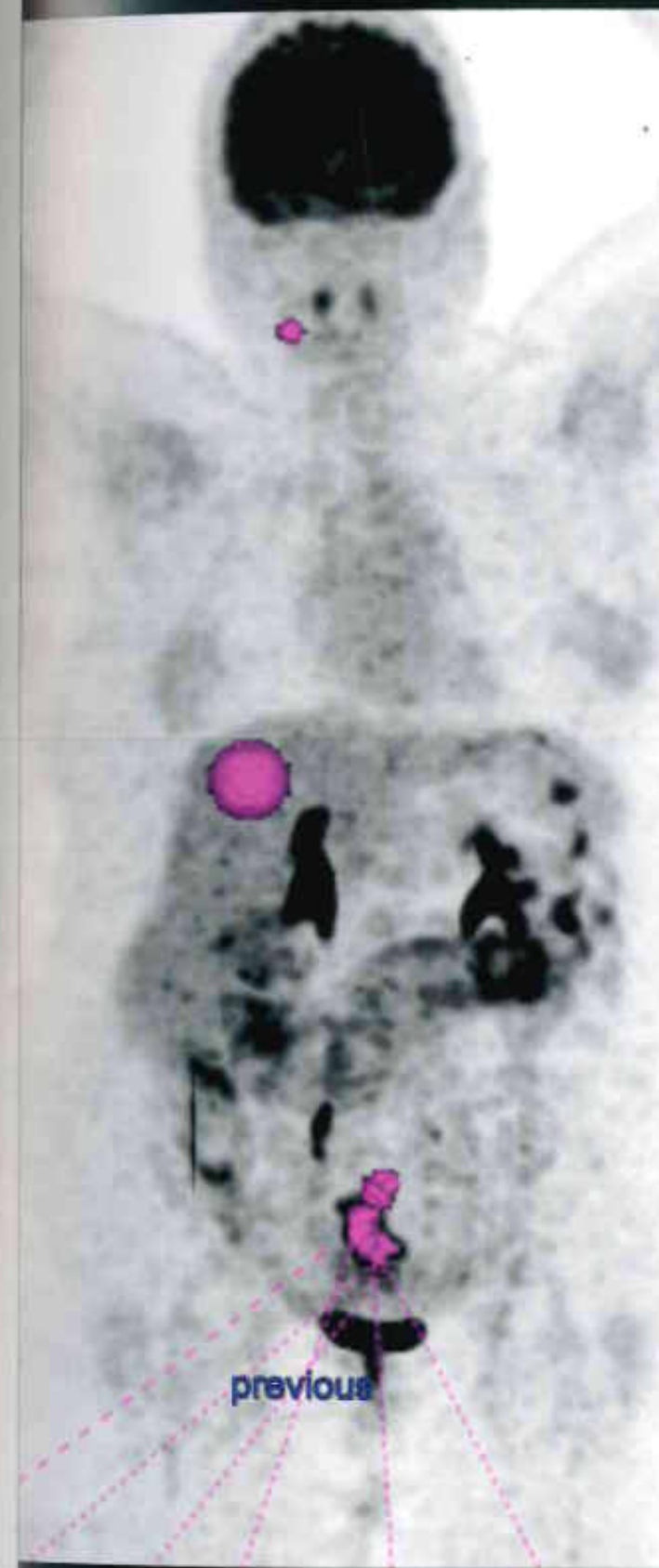


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Visit Number:	61126517456	Client Name:	
Patient Name:	Amal Abdelhakeem Mahmoud	Referred By Prof:	
Age / Sex:	60 year / Female	Exam Date:	15 Jul 2026 00:00
Patient ID:	2140144159 Acc: 2044353	Printed:	7/16/2026 6:41:49 PM

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FOLLOW UP WHOLE BODY PET/CT EXAMINATION

HISTORY AND CLINICAL DATA:-

PATIENT DATA:-

- Female patient, 60 years old, Weight: 92 Kg, Blood Glucose level: 179 mg/dl.

RADIOPHARMACEUTICAL:-

- F-18-FDG 5.5 mCi injected I.V.

CLINICAL BACKGROUND:-

- Reported case of colo-rectal carcinoma, received 3 CTH sessions, last one was 3/2026. And received RTH, last session 1.5 month ago.

PURPOSE OF THE STUDY: -

- Assess response to therapy.

TECHNIQUE: -

- Approximately one hour following the IV injection of the radio-pharmaceutical, a total body PET-CT was acquired from the mid-thigh to the base of the skull with arms extended above the head. CT was acquired for the purpose of anatomic localization, attenuation correction and diagnostic purpose.
- Data was processed and displayed according to departmental protocol.
- Adequate liver and brain FDG uptake with adequately co-registered PET and CT images.
- Liver showed average SUV ~2.0.
- Liver showed max. SUV~2.6.

REPORT:-

- ✓ Compared to the last study done on 18/4/2026.

- ✓ Review of the PET, CT and the fused images revealed:-

I) Loco-regional:

- **Mild metabolic regression despite further activity extension** of the previously seen FDG avid recto-sigmoidal segmental non-uniform neoplastic mural thickening, **extending now more towards the sigmoid colon** & reaches 1.8 cm in max. thickness; with max. SUV~11.1 compared to max. SUV~ 16.2 in last study.
- Still noted small mild FDG avid peri-colic fat stranding; with associated peri-rectal & pre-sacral lymph nodes; with SUVmax 4.8 compared to SUVmax 3.6 (mild metabolic progression) and measure up to 2 cm.

II) Liver:

Visit Number:	61126517456	Client Name:	
Patient Name:	Amal Abdelhakeem Mahmoud	Referred By Prof:	
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PT

- **Newly developed** FDG avid two hepatic focal lesions seen at the caudate lobe, with SUVmax 7.8 and measuring up to ~1.6 cm.

III) THE REST OF THE BODY:

- No other FDG avid lesions seen all over the rest of the surveyed body.
- No Ascites. No FDG avid peritoneal soft tissue lesions.
- Average sized spleen showing normal FDG uptake with no metabolically active FDG avid focal lesions.
- Normal bone marrow FDG uptake. No FDG avid lytic/sclerotic osseous lesions.
- Clear both lungs. No FDG avid pulmonary masses or nodules.
- No FDG avid adrenal or pancreatic masses.

SIDE FINDINGS:

- C4/C5 right facet arthropathy.
- Mild spondylosis.
- Elastofibroma dorsi.
- Aorto-iliac atheromatous mural calcifications.
- Large bilateral elasto-fibroma dorsi.
- Small hiatus hernia.

IV) The rest of PET / CT study appears normal.

V) Normal physiological tracer bio-distribution is seen over cerebral hemispheres, heart, bowel, bone marrow and urinary tract.

CONCLUSION:

- Follow up PET-CT study for a reported case of colo-rectal carcinoma showing:
 - ✓ Mild metabolic regression despite further activity extension regarding the FDG avid recto-sigmoid malignant mural thickening.
 - ✓ Mild metabolic progression of the FDG avid peri-rectal/pre-sacral metastatic LNs.
 - ✓ Newly developed FDG avid metastatic hepatic focal lesions.

Prof. Ahmed Wafaie MD, FRCR
Dr. Aya Mustafa Mussa, MD

جمهورية مصر العربية
بطاقة حقن التلقيح



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عبد الحكيم محمود مسلم حسن

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